

Chapter 2

Care of Patients (COP)

Intent of the chapter

The organisation provides uniform care to all patients in various settings. The settings include care provided in out-patient units, daycare facilities, in-patient units including critical care units, procedure rooms and operation theatre. When similar care is provided in these different settings, care delivery is uniform. Written guidance, applicable laws and regulations guide emergency and ambulance services, cardio-pulmonary resuscitation, use of blood and blood components, care of patients in the critical care and high dependency units.

Written guidance, applicable laws and regulations also guide the care of patients who are at higher risk of morbidity / mortality, high-risk obstetric patients, paediatric patients, patients undergoing procedural sedation, administration of anaesthesia, patients undergoing surgical procedures and end of life care.

Pain management, nutritional therapy and rehabilitative services are also addressed to provide comprehensive health care.

The management shall have written guidelines for organ donation and procurement. The organ transplant programme ensures that it has the right skill mix of staff and other related support systems to ensure safe and high quality of care.

The delivery of care and services to the patients are coordinated and integrated by all healthcare providers.

The standards aim to guide and encourage patient safety as the overarching principle for providing care to patients.

SUMMARY OF STANDARDS

COP.1.	Uniform care to patients is provided in all settings of the organisation and is guided by written guidance.*
COP.2.	Emergency services are provided in accordance with written guidance, applicable laws and regulations.
COP.3.	Ambulance services ensure safe patient transportation with appropriate care.
COP.4.	The organisation plans and implements mechanisms for the care of patients during community emergencies, epidemics and other disasters.
COP.5.	Cardio-pulmonary resuscitation services are provided uniformly across the organisation.
COP.6.	Nursing care is provided to patients in the organisation in consonance with clinical protocols.
COP.7.	Clinical procedures are performed safely.
COP.8.	Transfusion services are provided as per the scope of services of the organisation, safely.
COP.9.	The organisation provides care in intensive care and high dependency units in a systematic manner.

COP.10.	Organisation provides safe obstetric care.
COP.11.	Organisation provides safe paediatric services.
COP.12.	Procedural sedation is provided consistently and safely.
COP.13.	Anaesthesia services are provided in a consistent and safe manner.
COP.14.	Surgical services are provided in a consistent and safe manner.
COP.15.	The organ transplant programme is carried out safely.
COP.16.	The organisation identifies and manages patients who are at high risk of morbidity / mortality.
COP.17.	Pain management for patients is done in a consistent manner.
COP.18.	Rehabilitation services are provided to the patients in a safe, collaborative and consistent manner.
COP.19.	Nutritional therapy is provided to patients consistently and collaboratively.
COP.20.	End-of-life care is provided in a compassionate and considerate manner.

This implies that the objective element requires documentation.

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a	Commitment	Commitment	Commitment	Commitment	Commitment	Commitment	Commitment	Commitment	Commitment	Commitment
b	CORE	Achievement	Commitment	Commitment	Commitment	Commitment	Commitment	Commitment	Commitment	Commitment
c	Commitment	CORE	Commitment	Commitment	Commitment	Achievement	Commitment	Commitment	Commitment	Commitment
d	Excellence	Commitment	Commitment	Commitment	Commitment	Commitment	CORE	CORE	Commitment	Commitment
e	Excellence	Commitment	Commitment		Commitment	Commitment	Commitment	Commitment	Commitment	Achievement
f	Commitment	Commitment	Commitment		Commitment	Commitment	Commitment	Achievement	Achievement	Commitment
g		Commitment	Achievement				Commitment	Achievement	Commitment	Commitment
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j										Commitment
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a	Commitment	Commitment	Commitment	Commitment	CORE	CORE	Commitment	Commitment	Commitment	Commitment
b	Commitment	Commitment	Commitment	Commitment	Commitment	CORE	Commitment	Commitment	Commitment	Achievement
c	Commitment	Commitment	CORE	Commitment	Commitment	CORE	Commitment	Commitment	Commitment	Commitment
d	Commitment	Commitment	Commitment	CORE	CORE	CORE	Commitment	Commitment	Commitment	Commitment
e	Commitment	Commitment	CORE	Commitment		Commitment		Commitment	Commitment	Commitment
f	Commitment	Commitment	Commitment	Commitment				Commitment		
g	Commitment	Commitment	Commitment	Commitment				Excellence		
h	Excellence	Commitment	Commitment	Commitment						
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Standards and Objective Elements

Standard

COP.1.

Uniform care to patients is provided in all settings of the organisation and is guided by written guidance.

Objective Elements

Commitment a. Uniform care is provided to patients following written guidance . *

Interpretation: The organisation needs to implement mechanisms to ensure that the care, the patient receives in out-patient/day care/in-patient/emergency is uniform. Uniform patient care is linked to access, appropriateness of care and treatment. It does not depend on the patient's ability to pay or the source of payment. The level of care provided to patients shall be the same throughout the organisation.

Care delivery is uniform for a given clinical condition when similar care is provided in more than one setting. When similar treatment/care is provided in various settings such as out-patients, day care, in-patients, and different categories of wards, the organisation shall ensure that patients with the same clinical condition and care needs receive the same quality of health care throughout the organisation. Care delivery shall be applicable irrespective of the setting and category of the ward, and whether the patient is paying or non-paying, and/or is supported from a governmental/private insurance scheme or not. For example, the decision to offer any form of intervention or medication, frequency of consultant visit, nature of support care, decision to discharge is not influenced by the class or category of the patient but is decided by the clinical needs of the patient. Further, in case the organisation has separate OPDs for different category of patients, the methodology for care delivery shall be uniform in all OPDs.

Refer to AAC.4., AAC.5. and PRE.4.

CORE

b. The organisation has a uniform process for identification of patients and uses at least two identifiers.

Interpretation: The mechanism for identification of patients shall be uniform across the organisation. For example, the use of ID bands with the patient's name and unique identification number. The same shall be used in imaging, diagnostic and therapeutic procedures, blood transfusion, day care and nutrition/diet. For any care related aspect, at a minimum, two identifiers shall be used. One of the identifiers shall be the unique identification number generated at the time of registration.

Refer to AAC.2.b.



Commitment c. The organisation implements evidence-based clinical practice guidelines and / or clinical protocols to guide uniform patient care.

Interpretation: Clinical practice guidelines brought out by National and International professional organisations may be used. Standard treatment guidelines (STGs) brought out by the Government of India are a good starting point. In the absence of evidence-based clinical practice guidelines or where adapting the clinical practice guidelines are not feasible, sound clinical practices shall guide the delivery of care.

For definitions of “evidence-based medicine” and “clinical practice guidelines”, refer to the glossary.

Excellence d. Clinical care pathways are developed, consistently followed across all settings of care, and reviewed periodically.

Interpretation: Clinical care pathways shall be multidisciplinary, and based on the evidence and/or best clinical practices which provides detailed guidance at various stages of care. At a minimum the organisation shall develop two clinical care pathways every year. The clinical care pathways shall be reviewed and updated annually.

Excellence e. Multi-disciplinary and multi-speciality care, where appropriate, is planned based on best clinical practices / clinical practice guidelines and delivered in a uniform manner across the organisation.

Interpretation: Whenever the patient's clinical condition warrants care from multiple disciplines for example doctors, nurses, rehabilitation specialist, dietician, psychologist, etc and/or multiple specialties for example cardiology, endocrinology, surgery, internal medicine; multidisciplinary and multi-speciality care plan shall be charted out based on established best clinical practices/clinical practice guidelines for the specific clinical condition. An integrated care plan, including medical professional, nursing, nutritional, supportive care professionals, shall be developed and implemented appropriately by the representatives of various disciplines and specialities.

Some examples of scenarios where multidisciplinary care plan is required are care of a cancer patient to be determined by a multi-disciplinary tumour board; care of transplant patients; diabetes patient to be cared for by multiple specialists to prevent and manage end-organ damage; palliative/end-of-life care to be provided to terminally ill patients; long-stay patients.

Refer to PRE.3.e.

- Commitment f. Telemedicine facility is provided safely and securely based on written guidance. ***

Interpretation: Whenever the telemedicine facility is provided, the organisation shall develop written guidance in consonance with prevailing laws and guidelines and implement the same. The written guidance shall include protection of the patient's identity and confidentiality. The limitations of information and communication technologies shall be explicitly addressed. The organisation shall have a mechanism for appropriate data storage and retrieval.

The organisation shall have an MoU for its telemedicine services if outsourced.
Reference Telemedicine Practice ICMR Guidelines, 2020, MoHFW

Standard

COP.2.

Emergency services are provided in accordance with written guidance, applicable laws and regulations.

Objective Elements

- Commitment a. There shall be an identified area in the organisation which is easily accessible to receive and manage emergency patients, with adequate and appropriate resources.**

Interpretation: The identified area to treat emergency patients shall be easily accessible for the initiation of care. There shall be signage and directions in an understandable manner leading to the emergency area of the organisation. Emergency services preferably have designated triage, resuscitation treatment and patient holding areas identified. The organisation shall also specify the minimum number of beds based on its scope of services. At a minimum, adequate and appropriate human resource, basic resuscitation equipment, equipment for monitoring vital parameters, appropriate consumables and life-saving and emergency care drugs shall be available.

The personnel operating the emergency area shall be privileged to work in this area and have access to ongoing training programme.

Refer to FMS.2.d., HRM.5., HRM.6., HRM.11., HRM.12. and HRM.13.

Achievement b. Prevention of patient over-crowding is planned and crowd management measures are implemented.

Interpretation: Prevention of patient overcrowding in the emergency department includes monitoring the footfall trends, developing a strategy to manage overcrowding and violence. It is expected that high footfall times will be anticipated, and adequate human resources assigned and strategies to manage overcrowding are put in place.

Crowd management shall include having an appropriate policy for patient relatives/attendants/visitors.

CORE

c. Emergency care is provided in consonance with statutory requirements including medico-legal cases and as per written guidance. *

Interpretation: Written guidance shall include guidelines/SOPs/protocols to provide general emergency care as well as management of specific conditions, for example poisoning, road traffic accidents, patients with acute stroke and coronary disease, etc. It shall address both adult and paediatric patients. The procedure shall incorporate at a minimum identification, assessment and provision of care. In case, emergency services are out of the scope of the organisation, or the organisation does not have facilities for appropriate emergency care of a given clinical condition, at a minimum, such patients shall be provided with first aid before transferring them to another organisation.

Processes shall be in place to ensure patient safety.

The organisation shall also define as to what constitutes a medico-legal case (MLC). The care provided, especially the documentation and intimation to appropriate authorities, shall be in accordance with statutory requirements. The organisation shall have policy on management of suspected sexual assault and guidance on storage of samples of MLC patients.

Refer to COP.1.a-e.

Commitment d. Initiation of appropriate care is guided by a system of triage. *

Interpretation: Triage shall be done only by qualified/trained personnel. Written guidance based on evidence/sound clinical practices shall guide these activities. The triage shall be part of the routine day-to-day functioning of the emergency department and not from the perspective of managing a large number of patients during a disaster. The criteria could be separate for trauma and non-trauma patients and adults and children, for example, paediatric assessment triangle (PAT) / paediatric observation priority Score (POPS). If several clients are waiting to be triaged, a visual triage assessment may be conducted.

For "triage" refer to the glossary.

Commitment e. Patients waiting in the emergency are re-assessed as appropriate for the change in status.

Interpretation: A patient's condition may worsen or improve, and so a reassessment is needed for early identification of deterioration or improvement, and modification of care accordingly. The findings of reassessment shall be documented in the patients' medical record.

Commitment f. Admission, discharge to home or transfer to another organisation is documented.

Interpretation: The organisation shall maintain documentation to indicate if a patient who came to the emergency was sent home after providing initial care/was admitted for further care in the organisation/admitted in an emergency for a short stay and then discharged/transferred to another organisation. The staff shall have a clear understanding of the scope of the activities of the organisation and the procedure of referral and transfer to an appropriate another centre, of patients who cannot be cared for in-house, after administering the due first-aid/emergency care.

Refer to AAC.3.b.

Commitment g. In case of discharge to home or transfer to another organisation, a discharge / transfer note shall be given to the patient.

Interpretation: The discharge/transfer note shall contain salient clinical findings, investigations done, treatment given and condition at discharge/transfer. The reasons for discharge or transfer shall be documented.

Achievement h. The organisation shall implement a quality assurance programme. *

Interpretation: The written guidance for quality assurance shall be developed individually, or it could be a part of the organisation's overall quality-improvement programme. The quality assurance programme shall involve all aspects of the functioning in the Emergency department. The Emergency department shall collect data on key performance indicators, including care outcomes, as part of its quality improvement programme. An example of a care outcome measure is return to the emergency department for the same complaint.

The collected data shall be collated, analysed and used for further improvements. The improvements shall be monitored for sustenance.

Commitment i. The organisation has systems in place for the management of patients found dead on arrival and patients who die within a few minutes of arrival. *

Interpretation: There is written guidance for managing situations where a patient is either found dead on arrival or dies within a few minutes of arrival (after a failed attempt at resuscitation). The written guidance shall conform to the relevant local laws.

The written guidance in case of a patient found dead on arrival (brought in dead) to the emergency department address:

- Maintaining a logbook of patients found dead on arrival.
- The decision on whether to perform a post-mortem.
- The decision regarding the issue of medical certificate of cause of death.
- The temporary storage of the body in appropriate conditions.
- What to do in case of unclaimed/unaccompanied bodies.

In case of death of a patient within a few minutes after arrival (after a failed attempt at resuscitation), the written guidance shall address:

- Process of registration of such patients and recording the entire resuscitation events.
- The decision on whether to perform a post-mortem.
- The temporary storage of the body in appropriate conditions.
- Issue of Medical certificate of cause of death and handing over of the body.

Standard

COP.3.

Ambulance services ensure safe patient transportation with appropriate care.

Objective Elements

Commitment a. The organisation has access to ambulance services commensurate with the scope of the services provided by it.

Interpretation: Commensurate to its scope of services, the organisation may provide in-house or use out-sourced ambulance service for safe patient transport with appropriate care.

The organisation shall decide the appropriate level of an ambulance to be provided based on the National Ambulance Code AIS-125.

Commitment b. There are adequate access and space for the ambulance(s).

Interpretation: The organisation shall demarcate a proper space for the

ambulance(s). The demarcation shall be done keeping in mind easy accessibility for receiving patients and enabling the ambulance(s) to exit quickly. Adequate and prominent signage shall exist to guide the ambulance drivers to the ambulance entry and route to the emergency department. It is preferable that the organisation has an ambulance parking bay.

Commitment c. The ambulance(s) is fit for purpose and is appropriately equipped.

Interpretation: The vehicle used as an ambulance shall adhere to statutory requirement for example registration as an ambulance under the Motor Vehicle Act, valid fitness certificate, pollution control certificate, and insurance of the vehicle.

An ambulance shall have at least basic life support equipment, for both adult and paediatric patients. Based on the organisation's scope, additional equipment, for example monitoring and resuscitative equipment, shall be available in the ambulance.

Refer to National Ambulance code AIS-125.

Commitment d. The ambulance(s) is operated by trained personnel.

Interpretation: The ambulance shall be operated by a driver with a valid driving licence. Additionally, technician/nurse and/or doctor may be there depending on the situation and scope of the ambulance. Personnel in the ambulance shall have training in basic life support and be trained in basic cardiopulmonary resuscitation.

Commitment e. The ambulance(s) is checked daily for functioning status, medical equipment, emergency medications and consumables.

Interpretation: The check shall indicate the functioning status of the ambulance like lights, siren, beacon lights, fuel, tyres etc.

The checks shall indicate the functioning status of the medical equipment based on a documented checklist.

Emergency medications are available in the ambulance during patient transport. Based on a checklist, a daily check is done to ensure availability and expiry dates of emergency medications and documented. After the return from every trip, medications used shall be topped up if used and the same verified and documented. The medications shall be stored in a safe environment as per policy of the organisation. In case the organisation follows a system of sealing the emergency medication kit, then the check shall be carried out after each use of the kit.

Commitment f. The ambulance(s) has a proper communication system.*

Interpretation: The ambulance shall be connected with the organisation/control room by wireless/mobile phones. The communication system shall encompass the whole process of patient transport. There shall be written guidance by the organisation as to how a call for patient transport is received, who are the people expected to respond and organise the transport. The communication ensures that the ambulance leaves the hospital within a predefined timeframe based upon the patient's needs.

Achievement g. The emergency department identifies opportunities to initiate treatment at the earliest when the patient is in transit to the organisation.

Interpretation: From the time of first communication with the patient/ patient's attendant, a file is created to record appropriate information, attempts are made to gather important clinical information (patient's age, weight, provisional diagnosis and ongoing treatment at the referring organisation). This information is used by the ambulance personnel of the receiving organisation to be better prepared to assess, initiate emergency care/interventions during transit and transport the patient safely. The organisation shall ensure to focus on pre-hospital emergency care for seriously ill or injured patients before they reach hospital, and during emergency transfer to organisation or between organisations.

During the transit, when required, there is an exchange of information between the ambulance personnel and the medical professional at the receiving organisation. The information will help the doctor at the receiving organisation guide the ambulance personnel to facilitate the management during the transit.

When the patient is being shifted by an external agency, wherever possible, an attempt is made by the doctor of the receiving organisation to communicate with the ambulance personnel of the external agency to ascertain the clinical situation and make appropriate suggestions. However, the medical professional in the ambulance would be responsible for decision-making regarding the care/interventions during the transit.

Standard

COP.4.

The organisation plans and implements mechanisms for the care of patients during community emergencies, epidemics and other disasters.

Objective Elements

- Commitment a. The organisation identifies potential community emergencies, epidemics and other disasters.***

Interpretation: The organisation shall identify potential community emergencies, epidemics and other disasters likely to cause a sudden rush of victims. Some examples include earthquake, floods, train accident, civil unrest outside the organisation's premises, major fire and outbreak of disease/epidemics.

These shall be identified based on geographical location and the community served by the organisation. For example, an organisation in an industrial town shall identify the industrial hazards that may occur in its vicinity.

- Commitment b. The organisation manages community emergencies, epidemics and other disasters as per a documented plan.***

Interpretation: The disaster, community emergency and epidemic plan must incorporate essential elements of alert code, information and communication, action cards for each of the staff, availability and earmarking of resources including adequacy of medical supplies, equipment, materials, trained personnel, establishment of command nucleus, training and mock drills, and managing clinical activities during the event. The emergency room could follow triage policy according to the National Disaster Management Authority (NDMA) guidelines.

It shall also include aspects like activating and deactivating plan; receive, identify and triage casualties; defined areas for reception and treatment for casualties; transportation aids; communication aids; manage visitors, and control the movement of individuals and vehicles, relocate/discharge admitted patients wherever needed.

The plans shall conform to the relevant local laws and national plans on disaster management. A good reference is NDMA guidelines.

- Commitment c. Provision is made for availability of medical supplies, equipment and materials during such emergencies.**

Interpretation: Resource availability shall be according to threat perception. The number of resources i.e. medical consumables, equipment, etc. to be commensurate with the expected workload.

Commitment d. The plan is tested at least twice a year.

Interpretation: Testing twice a year is only the minimum frequency, and this may be increased. In case the organisation has different plans for different disasters, each of the plans shall be tested at least twice a year.

The plan can be tested using a table-top exercise, or a mock drill. At a minimum, at least one mock drill shall be held once in 12 months. This shall test all the components of the plan and not just awareness. In the case of a mock drill, simulated patients (not real) shall be used. After every table-top exercise/mock drill, the variations are identified, the reason for the same is analysed, debriefing conducted and where appropriate the necessary corrective and/or preventive actions are taken.

Standard

COP.5.

Cardio-pulmonary resuscitation services are provided uniformly across the organisation.

Objective Elements

Commitment a. Cardio-pulmonary resuscitation services are available and provided to patients at all times.

Interpretation: The organisation shall document the procedure for cardio-pulmonary resuscitation (CPR) for adults across all areas in the organisation. This shall be in consonance with accepted practices. Where appropriate, it shall also address obstetric, paediatric and neonatal patients. The organisation shall ensure that medical equipment for resuscitation and medications for basic and advanced life support are provided in standardised manner. Basic life support shall be initiated as soon as a condition requiring CPR is identified. This shall be implemented in all areas of the organisation. The protocols could be displayed prominently in all critical areas such as emergency, ICU, OT and all crash carts etc.

Commitment b. During cardio-pulmonary resuscitation, assigned roles and responsibilities are complied with.

Interpretation: The CPR team members have a clear understanding of their roles and responsibilities during the resuscitation to effectively function as a team.

Commitment c. Medical equipment and medications for use during cardio-pulmonary resuscitation are available in various areas of the organisation.

Interpretation: At a minimum, emergency medications and equipment for intubation based on the needs of the patients served shall be available in patient care areas including the blood centre, radiology, day care, dialysis, chemo ward, OPD, rehabilitation services areas, endoscopy, and in areas where any invasive procedure is performed. Other equipment like defibrillator shall be easily accessible to ensure that there is no delay in cardio-pulmonary resuscitation. It is preferable that the minimum emergency medication is standardised across the organisation.

Commitment d. The events during cardio-pulmonary resuscitation are recorded.

Interpretation: In the actual event of cardio-pulmonary resuscitation, or a mock drill of the same, all the activities along with the personnel attended shall be recorded. At the minimum, it will include timeliness of response, availability of human resources, equipment, drugs, and barriers if any. The recording could be done using the pre-defined procedural checklist and by monitoring whether the prescribed activity has been performed properly and in the right sequence.

It is a good practice to debrief team members regarding the necessary immediate corrective and preventive action.

Commitment e. A multi-disciplinary committee does a post-event analysis of cardio-pulmonary resuscitations.

Interpretation: The frequency of the committee meeting shall be at least once a quarter. The analysis shall focus on the initiation of CPR, time of arrival of the team, availability of required resources, recording of the sequence of events during CPR (including technique) and the overall coordination. The organisation shall monitor outcome of CPR and identifies areas for improvement. The multidisciplinary committee shall be independent and include at least one physician/cardiologist, anaesthesiologist, one member from the code blue team and nurse. The analysis shall be completed within a defined time frame.

- Commitment f. Corrective and preventive measures are taken based on the post-event analysis.**

Interpretation: Corrective and preventive measures shall be completed within a defined time frame. The findings of the post-event analysis are communicated to the personnel participated in the CPR. Any lapses shall be discussed, with the view to improve the outcomes in future. During subsequent resuscitations, it is preferable that implementation of these actions is noted and training be modified, if necessary.

Standard

COP.6.

Nursing care is provided to patients in the organisation in consonance with clinical protocols.

Objective Elements

- Commitment a. Nursing care is provided to patients in accordance with written guidance.***

Interpretation: The written guidance could be in the form of a nursing manual/SOPs incorporating various basic nursing practices and procedures. Examples of these are monitoring vital parameters, administration of medications, basic hygiene needs of the patient, etc.

Care of patients in specific clinical situations shall be guided by nursing clinical practice guidelines based on best clinical practices. The nursing clinical care guidelines/pathways shall be reviewed annually at the minimum, and revised as appropriate.

Examples of nursing clinical practice guidelines include prevention of fall, prevention of development of pressure ulcers in an in-patient, and deep venous thrombosis risk assessment and prevention.

- Commitment b. Assignment of patient care is done as per good clinical / nursing practice.**

Interpretation: Assignment shall be based on the patient's clinical requirements, the competence of the nursing staff, and shall align with the guidelines laid down by regulatory and professional bodies in this regard.

Achievement c. The organisation implements acuity-based staffing to improve patient outcomes.

Interpretation: Patient outcomes are linked to acuity-based staffing of nursing personnel, in terms of numbers and competence. Examples of outcomes include incidence of pressure sores, falls, medication administration errors, ventilator-associated pneumonia, etc.

Commitment d. Nursing care is aligned and integrated with overall patient care which is documented.*

Interpretation: Care shall be provided as per the nursing care plan, which is individualised as per the clinical needs of the patient. Wherever a patient care plan has been developed, the nursing care plan shall be aligned with the same. Uniformity and continuity of care shall be practised.

Components of the nursing care plan include:

- Assessment
- Plan of care
- Implementation of care
- Evaluation
- Modification of plan of care as may be required

The documentation includes all nursing-related care and not just monitoring of vitals and documentation of medication administration. The nursing progress shall be documented in a timely manner for the individual patient.

Commitment e. Nurses are provided with the appropriate and adequate equipment for providing safe and efficient nursing care.

Interpretation: There shall be an adequate number of nebuliser machines, glucometers, sphygmomanometers, thermometers, weighing scale(s), and other basic equipment/gadgets necessary for functioning in the designated area. Further, the equipment shall be appropriate for the area. For example, the BP cuffs in the Paediatric area shall be of appropriate size.

Commitment f. Nurses are empowered to make patient care decisions within their scope of practice.

Interpretation: The organisation shall define the patient care decisions that come under the scope of nursing practice. Nurses shall be aware of the same and be able to make appropriate nursing-related decisions in a timely manner.

Standard

COP.7. Clinical procedures are performed in a safe manner.

Objective Elements

Commitment a. Clinical procedures are performed based on the clinical needs of the patient.

Interpretation: The decision to perform a procedure shall be based on the clinical needs of the patient, in consonance with standard treatment guidelines and/or sound clinical practice for the given condition/procedure. A qualified medical practitioner decides if the procedure is indicated. When multiple procedure options exist, the decision shall be based on which option is likely to provide the best outcome, and also taking into consideration, the patient wishes and safety. The organisation could conduct a clinical audit of various procedures to achieve the best possible outcomes.

Commitment b. Performance of various clinical procedures is based on written guidance and done in a safe manner. *

Interpretation: The written guidance is a broad guideline applicable to all procedures - diagnostic, therapeutic, and supportive. The written guidance shall incorporate as to who will do the procedure, the pre-procedure instructions where applicable, the conduct of the procedure and post-procedure instructions and care, where applicable.

The organisation shall establish and implement safe use of equipment used for performing procedures and treatments. A brief assessment, including at least patient's vitals is done prior to performing the procedure.

Commitment c. Qualified personnel order, plan, perform and assist in performing procedures.

Interpretation: The personnel performing or assisting a procedure shall be privileged for the same.

Refer to HRM.11.d., HRM.12.d. and HRM.13. c.

CORE

d. Care is taken to prevent adverse events like a wrong patient, wrong procedure and wrong site. *

Interpretation: The organisation shall use a documented check-list to prevent adverse events like a wrong patient, wrong procedure and wrong site. This could be based on the WHO safety check-list or its modification. At least two identifiers shall be used to identify the patient out of which one shall be the unique identification number. Besides, the organisation shall have a procedure to identify the site of the procedure, where appropriate. The organisation identifies those invasive procedures within its scope where a pre-procedure checklist could be used to mitigate the risk of wrong-site/side, wrong patient and wrong procedure.

The responsibility for ensuring the correct site and side where applicable/patient/procedure verification rests with all team members. However, the person performing the procedure carries ultimate responsibility. In case the procedure is being performed by a person in training, the supervising clinician carries ultimate responsibility.

In emergencies, all attempts shall still be made to identify the correct site (including side where applicable)/patient/procedure according to the laid down guidance, although it may not be possible or appropriate to complete all the checks. Any exceptions to the full protocol shall be documented in the medical record.

Patient and/or relatives shall be involved during site marking in ensuring correct patient, correct procedure and correct site whenever possible.

Refer to AAC.2.b. and COP.1.b.

Commitment e. Informed consent is taken by the personnel performing the procedure, where applicable.

Interpretation: The informed consent shall be taken by the person performing the procedure or a doctor from the treating team. In case the procedure is being done by a person in training, it shall specify the same and shall be supervised by the treating doctor.

Refer to PRE.4.e.

Commitment f. Patients are appropriately monitored during and after the procedure.

Interpretation: At a minimum, for invasive procedures, this shall include pulse, blood pressure and respiratory rate and any other parameter as clinically required. The extent and duration of monitoring may be tailored to the need based on the complexity of the procedure and the co-morbidities of the patient.

Commitment g. Procedures are documented accurately in the patient record.

Interpretation: The documentation shall mention the name of the procedure, the person who performed the procedure, salient steps of the procedure, key findings and the post-procedure care. All documentation shall have name, date, time and signature.

Refer to IMS.3.e.

Standard

COP.8.

Transfusion services are provided as per the scope of services of the organisation, safely.

Objective Elements

Commitment a. Scope of transfusion services is commensurate with the services provided by the organisation.

Interpretation: The organisation shall have blood/blood components available from either an in-house or out-sourced registered blood centre.

The blood centre shall have adequate resources (infrastructure, manpower and equipment) as per workload, scope in order to ensure the smooth functioning of its operations and to avoid delays.

In case the organisation uses an out-sourced blood centre, it shall have an MoU and ensure that patient care does not suffer for want of blood/blood components. The blood shall be transported from the external blood centre safely and properly. A good reference guide is the NABH standards for blood centre.

Commitment b. The organisation shall establish and implement processes for blood / component collection, testing, storage and distribution under written guidance.*

Interpretation: The blood centre shall use written guidance for procedures pertaining the following activities:

- blood donor selection;
- blood screening for transfusion transmissible diseases;
- blood collection including apheresis procedures;
- blood storage;
- compatibility testing;
- blood distribution;
- transfusion of blood/products in clinical areas
- discard of sero positive / unutilised blood; and
- education of family (potential donors) and patient regarding blood donation

Blood/apheresis products shall be collected from voluntary, non-remunerated, low risk, safe and healthy donors with due emphasis on pre-donation and post-donation counselling (including consent) and procedures and the informed consent process. Apheresis procedures shall be carried out as per guidelines while ensuring the quality of the product. Blood donation drives shall be organised as per national and statutory guidelines.

Necessary drugs and equipment shall be available for treatment of donor reaction, if any, and staff is trained on using the same. The blood centre shall have mechanisms to identify, manage and report adverse donor reactions in line with the National Haemovigilance Programme of India (NHvPI).

All pre-transfusion tests including Transfusion Transmissible Infections (TTI) and Red Cell Serology are carried out by an approved methods before release of blood.

Blood centre shall retain and store recipient's and donor's blood samples after each transfusion as per guidelines.

In event of quality control failure, in case of TTI testing, defined procedures shall ensure that quarantined units are not taken into inventory till re-testing is completed.

The Blood Centre shall have a process for retesting, recall and referral of reactive blood donors in line with national guidelines.

The whole blood or components from any unit that tests positive/ reactive shall be discarded as per written guidance.

Commitment c. Blood and components are stored safely from the time of collection till transfusion.

Interpretation: The Blood Centre shall prepare components as per their scope. The blood centre shall ensure that the temperature of whole blood and its component and the environment are maintained and recorded as per guidelines. Untested blood/products shall be quarantined with designated and separate storage of untested and tested units for their serology status. Labelling (inclusive of serology status) of blood/components shall be done after the reports of TTI screening are released to act as a check against untested blood/products getting mixed with tested blood/products.

CORE d. The organisation ensures safe and rational use of blood and blood components. *

Interpretation: Transfusion of blood and blood components is managed by written guidance to ensure recipient safety. The written guidance shall include

- patient consent for blood administration (Refer to PRE.4.);
- safe procurement and transportation (from blood centre or outsourced blood centre) while maintaining the cold chain and delivery at the right source;
- patient identification;
- verification of blood administration orders;
- safe storage as per requirements, prior to transfusion;
- blood administration;
- monitoring of patient;
- identification and responding to potential transfusion reactions (Refer to COP.8.h.).

Measures shall be taken by the organisation to prevent the possibility of mismatched transfusion, under all situations. For example,

- correct patient identification at the time of cross-matching,
- the compatibility testing label is attached to the unit at the time of issue,
- correct patient identification and verification of patient and donor blood group at the time blood transfusion.
- Ensuring that a portion of the integral tubing together with segment number remains attached to blood bag at the time of issue from blood centre.
- Ensuring all labels shall continue to remain attached to the blood bag till the completion of transfusion to facilitate appropriate investigation of an adverse transfusion reaction when required.

Blood and blood component usage shall be governed by written guidance which shall address the indications for the use of blood and blood components. These shall be based on the standard practice guidelines/sound clinical practices brought out by the national and international professional organisations. It shall also address inventory and ordering schedules (planned and unplanned). The blood centre protocols could include specific clinical situations, based on the scope of services of the organisation, for example, neonatal transfusions, auto-immune haemolytic anaemia, incompatible transfusion, massive transfusion.

A Hospital Blood Transfusion Committee (HBTC) shall ensure the rational use of blood at the organisation by adopting/adapting guidelines for the rational use of blood/products. The committee shall preferably be chaired by a clinician and shall have representation from different clinical departments, blood transfusion officer, Nursing, quality coordinator and the management. The committee shall meet periodically and shall monitor the availability of blood/components, transfusion practices and their audits, adverse transfusion reactions, wastage of blood/products etc. The committee could also obtain feedback from clinicians periodically to assess the availability and efficacy of blood/components.

Commitment e. Blood / blood components are available for use in emergency and routine situations within a defined time-frame. *

Interpretation: The organisation shall define and document as to what constitutes “use in an emergency situation” and accordingly develop procedures to ensure the availability of blood/blood components.

The organisation shall define the time frame within which blood shall be available for use in an emergency situation. Use in an emergency includes both actual and anticipated need.

Turn-around time for routine and emergency issues of blood/product is defined and monitored.

The objective element is applicable even if the organisation doesn't have the blood center facility in-house.

Achievement f. The organisation shall ensure that post-transfusion form is collected, reactions if any identified and are analysed for preventive and corrective actions. *

Interpretation: The organisation shall ensure detection, reporting, evaluation and analysis of suspected adverse transfusion reactions followed by necessary corrective and preventive action.

It is preferable that the organisation capture feedback regarding every transfusion as this would enable it to capture all transfusion reactions. These are then analysed (by individual/committee as decided by the organisation), and appropriate corrective/preventive action is taken. The organisation shall maintain a record of transfusion reactions.

The organisation shall participate in the Haemovigilance Programme of India.

Refer to IMS.1.g.

For “transfusion reactions” refer to the glossary.

Achievement g. The organisation shall implement a quality assurance programme. *

Interpretation: The written guidance for quality assurance could be developed individually, or it could be a part of the organisation’s overall quality-improvement programme. The quality assurance programme shall involve all aspects of the functioning in the blood centre.

The blood centre participates in External Quality Assurance Scheme (EQAS)/ Proficiency Testing Programme (PT) or a suitable interlaboratory comparison when the same is not available.

Results of IQC/EQAS/PT/Inter lab testing are monitored and appropriate corrective and preventive action taken for continual quality improvement and records of the same maintained.

Appropriate sampling of blood/products and reagents shall be done at defined frequency for internal quality control, as per guidelines.

The blood centre shall collect data on key performance indicators as part of its quality improvement programme. An example of a process measure is wastage of blood and blood products and an example of outcome measure is transfusion reactions.

The collected data shall be collated, analysed and used for further improvements. The improvements shall be monitored for sustenance.

Standard

COP.9.

The organisation provides care in intensive care and high dependency units in a systematic manner.

Objective Elements

- Commitment a. Care of patients in intensive care and high dependency units is provided based on written guidance. ***

Interpretation: The written guidance shall be based on standard treatment guidelines/sound clinical practices regarding intensive/critical care. At a minimum, this shall address all the objective elements of this standard. Where appropriate, the organisation could adapt care-bundles.

- Commitment b. The defined admission and discharge criteria for intensive care and high dependency units are implemented. ***

Interpretation: The organisation shall develop criteria for admission/transfer-in/discharge/transfer-out of patients to ICU/HDU based on physiologic parameters and/or diagnostic parameters, and adhere to the same. The organisation shall have a mechanism for "discharge against medical advice". A good starting point could be various national and international critical care society guidelines.

Staff are trained to apply these criteria.

- Commitment c. Adequate staff and equipment are available.**

Interpretation: Statutory requirements, wherever applicable, shall be adhered. The ICU shall be equipped with all necessary lifesaving and monitoring equipment as well as suitably handled by trained staff. The exact requirements shall be decided by the organisation based on the scope and complexity of its services. However, the organisation is expected to follow standard treatment guidelines/best clinical practices.

Staff working in ICU/HDU shall be competent (based on qualification and/or training).

A good reference guide for the nursing workforce is the Indian Nursing Council recommendations.

- Commitment d. Defined procedures for the situation of bed shortages are followed. ***

Interpretation: As and when there are no vacant beds in the ICU, and there is a requirement of such bed, detailed policy and procedure shall be in place to address the situation.

Commitment e. Infection prevention and control practices are followed. *

Interpretation: Written guidance for infection prevention and control could be developed individually or could be a part of the infection prevention and control manual. The organisation shall ensure that the practices are in consonance with standard treatment guidelines/sound clinical practices.

Refer to IPC.3.

Achievement f. The organisation shall implement a quality assurance programme. *

Interpretation: The written guidance for quality assurance could be developed individually, or it could be a part of the organisation's overall quality-improvement programme. The quality assurance programme shall involve all aspects of the functioning in the intensive care unit. The programme shall be in consonance with standard treatment guidelines/sound clinical practices. The organisation shall monitor care outcomes, like risk-adjusted standardised mortality rate, infection rates, re-admission rates, re-intubation rates, etc. Further, a good starting point could be various national and international critical care society guidelines on quality assurance in ICUs.

Commitment g. The organisation has a mechanism to counsel the patient and/or family periodically.

Interpretation: Patients and families are counselled by a doctor of the treating team at periodic intervals (at least once a day) and when there is a significant change in the condition of the patient. Significant events since the last counselling session, expected outcomes, and queries related to the changing condition of the patient shall be addressed. Counselling shall be documented and/or audio video recorded. It is preferable to obtain an acknowledgement for the same.

Standard

COP.10.

Organisation provides safe obstetric care.

Objective Elements

Commitment a. Obstetric services are organised and provided safely. *

Interpretation: Written guidance based on standard treatment guidelines/sound clinical practices govern the organisation and delivery of safe obstetric care. At a minimum, this shall include assessment of pregnant patients incorporating nutrition, immunisation and education; and ante-natal, peri-natal and post-natal care guidelines.

The organisation shall define in its written guidance, whether care for high-risk obstetric cases is provided or not. The organisation shall define as to what constitutes a high-risk obstetric case in consonance with best clinical practices.

Staff shall be trained on the management of obstetric emergencies, based on evidenced-based clinical practice guidelines.

Care of the new-borns is in accordance with COP:11.b.

Commitment b. The organisation identifies and, provides care to high-risk obstetric cases, and where needed, refers them to another appropriate centre.

Interpretation: Ante-natal examination(s) shall guide the early identification of high-risk obstetric cases. High-risk obstetric cases shall have more frequent ante-natal check-ups, where appropriate. In case, care of high-risk obstetric cases is not in the scope of the organisation's obstetric services; there shall be a pro-active referral to the appropriate organisation.

The organisation caring for high-risk obstetric cases have competent workforce and facilities to take care of such mothers and neonates.

Commitment c. Persons caring for high-risk obstetric cases are competent.

Interpretation: These shall not just be doctors but shall include nursing staff also. The competency shall be based on qualification and/or experience and/or training.

Commitment d. Ante-natal services are provided. *

Interpretation: Written guidance governs ante-natal services. The guidance shall, at a minimum include assessment, immunisation, diet counselling and frequency of visits. There shall be an ante-natal card (or equivalent) for every such patient.

The antenatal card shall be complete with medical and obstetric history of the expectant mother.

Achievement e. Organisation encourages and welcomes the presence of a birth companion during labour.

Interpretation: The staff shall encourage and welcome the presence of a birth companion during labour.

Refer to MoHFW LaQshya guidelines 2017

Commitment f. Organisation treats pregnant woman and her companion cordially and respectfully, ensures privacy and confidentiality for pregnant woman during her stay.

Interpretation: There are provisions for privacy in the Labour / Delivery Room (curtains /partition between tables and non-see-through windows etc. Staff treat pregnant woman and her companion cordially and respectfully.

Refer to MoHFW LaQshya guidelines 2017.

Commitment g. The treating doctor explains danger signs and important care activities to pregnant woman and her companion.

Interpretation: The treating doctor or team member/ doctor shall explain danger signs and important care activities to the expecting mother and her companion.

Refer to MoHFW LaQshya guidelines 2017.

Commitment h. Obstetric patient's assessment also includes maternal nutrition.

Interpretation: Obstetric patient's assessment also includes maternal nutrition and it is documented in the ante-natal card (or equivalent) for every such patient. It is preferable that the assessment of maternal nutrition is done by a dietician.

Commitment i. Appropriate peri-natal and post-natal monitoring is performed.

Interpretation: This is in context of maternal and foetal monitoring. At a minimum, this shall include monitoring of foetal heart rate during labour, the progression of labour and post-natal monitoring for post-partum haemorrhage.

Commitment j. The organisation caring for high-risk obstetric cases have the facilities to take care of neonates of such cases.

Interpretation: The organisation shall have a NICU (level I, II or III) with appropriate equipment and staff, including competent paediatrician/neonatologist and nurses (based on qualification and training).

Commitment k. Organization shall adhere to legal and defined Assisted Reproductive Technology (ART) practices.

Interpretation: This shall include infrastructure, ART lab, operation theatre, a competent ART team counsellors and counselling techniques, consent process, screening of patients and SOPs as defined by national guidelines, and Assisted Reproductive Technology Act as amended.

Standard

COP.11. Organisation provides safe paediatric services.

Objective Elements

Commitment a. Paediatric services are organised and provided safely. *

Interpretation: Written guidance based on standard treatment guidelines/sound clinical practices govern the organisation and delivery of safe paediatric care. At a minimum, this shall include assessment of these patients, organisation of care, and addressing special needs.

The written guidance shall also define the scope of its paediatric services. The scope shall include various paediatric sub-specialities and special clinics. For example, well-baby clinics, different levels of NICUs, PICU etc.

Commitment b. Neonatal care is in consonance with the national / international guidelines. *

Interpretation: Written guidance based on standard treatment guidelines/sound clinical practices govern the delivery of neonatal care. These shall be in consonance with guidelines laid down by national and international bodies. The hospital shall actively promote breastfeeding practices.

Commitment c. Those who care for children have age-specific competency.

Interpretation: Doctors and nursing staff shall have age-specific competency. The competency shall be based on qualification, experience, and/or training.

Commitment d. Provisions are made for special care of children.

Interpretation: Adequate amenities for the care of infants and children to be available in the hospital. For example, breastfeeding room, play area, etc.

Commitment e. Paediatric assessment includes growth, developmental, immunisation and nutritional assessment.

Interpretation: The growth, developmental, immunisation and nutritional assessment is performed using appropriate tools (preferably validated) and documented. The growth charts and immunisation records are maintained and updated as applicable.

Commitment f. The organisation has measures in place to prevent child / neonate abduction and abuse. *

Interpretation: Written guidance shall direct the organisation regarding child abduction prevention and abuse. The organisation shall ensure that there is adequate security/surveillance to prevent such happenings. For example, the installation of CCTV cameras. There is a defined process for rapid response in case of any eventuality. The defined process shall be tested at pre-defined intervals, either through table-top exercise or mock drill. Staff are trained in prevention and rapid response. Staff is aware of how to handle and escalate child abuse, if any.

Commitment g. The child's family members are educated about nutrition, immunisation and safe parenting.

Interpretation: The education shall be in the language that the family understands. This could be done using educational material. The education on nutrition shall address the importance of breastfeeding, weaning and various aspects of malnutrition that includes protein-energy malnutrition and childhood obesity. Appropriate at home growth monitoring information is shared with parents.

Excellence

h. The organisation provides for adolescent friendly health care services.

Interpretation: Need based preventive, curative and counselling services for adolescents in hospital setting shall be provided. Hospital staff shall adopt a respectful and non-judgemental attitude during the entire process of registration, reception, consultation, treatment etc. Confidentiality shall be maintained in consultations, counselling, and follow-up. The adolescent health care service may be organised as follows:

- Management of physical and mental disorders.
- Counselling services including premarital counselling, anti-substance abuse, use of helmets while driving two wheelers wherever applicable.
- Preventive health check-ups and wellness program.
- Immunization, for example vaccine against HPV.
- Health education activities in the form of information brochure, videos, website, handouts etc this could be on normal development, nutrition, sleep, responsible sexual behaviour, safe sexual practices, substance use prevention etc.

Refer AAC.11.

Standard

COP.12.

Procedural sedation is provided in a consistent and safe manner.

Objective Elements

Commitment a. Procedural sedation is administered in a consistent manner. *

Interpretation: Written guidance based on standard treatment guidelines/sound clinical practices govern the administration of procedural sedation. At a minimum, this shall include identification of areas in the organisation where procedural sedation is administered and procedures are performed. Written guidance also includes the mechanism for writing orders, the pre-procedure assessment, monitoring during and after the procedure and the discharge/transfer out criteria after the procedure. A pre-sedation assessment shall be performed and documented to evaluate risk and appropriateness of procedural sedation for the patient. The scope and content of this assessment shall be based on professional guidelines and are defined in written guidance.

The administration of procedural sedation shall be standardised throughout the organisation.

Commitment b. Informed consent for administration of procedural sedation is obtained.

Interpretation: The informed consent shall be taken by the person administering sedation or a doctor member of the team administering sedation. The risks, benefits and alternatives related to procedural sedation shall be informed. The patient and family are educated about the post-procedural analgesia.

Refer to PRE.4.e.

Commitment c. Competent and trained persons administer procedural sedation.

Interpretation: Whenever a parenteral route is used, sedation may be administered by a doctor or a nurse under the supervision of a doctor. The technician shall not administer sedation.

Commitment d. The person monitoring sedation is different from the person performing the procedure.

Interpretation: The person responsible for monitoring the patient shall be trained in the detection of abnormalities of the monitoring parameters (Refer to COP.12.e.) and also in recognition of apnoea and airway obstruction.

Commitment e. Intra-procedure monitoring includes at a minimum the heart rate, cardiac rhythm, respiratory rate, blood pressure, oxygen saturation, and level of sedation.

Interpretation: The monitored parameters shall be documented. Also, certain other parameters may be monitored on a case-to-case basis. The cardiac rhythm may be monitored on a monitor during the procedure, and the same need not be documented. However, in the case of rhythm abnormalities, the same shall be documented.

Commitment f. Patients are monitored after procedural sedation, and the same is documented.

Interpretation: The patient's vitals shall be monitored at regular intervals (as decided by the organisation) until he/she recovers completely from the sedation. At a minimum, the heart rate, respiratory rate, blood pressure, oxygen saturation

and level of sedation are monitored. The extent and duration of monitoring may be tailored to the need based on the complexity of the procedure and the co-morbidities of the patient. The level of sedation can be monitored by using a checklist which incorporates the various components of levels of sedation (mild, moderate and deep).

Commitment g. Criteria are used to determine the appropriateness of discharge from the observation / recovery area. *

Interpretation: Criteria shall be developed and documented by the organisation in consonance with physiologic parameters and sound clinical practices. A qualified individual shall apply the criteria, and the same is documented.

Commitment h. Equipment and workforce are available to manage patients who have gone into a deeper level of sedation than initially intended.

Interpretation: The room where procedural sedation is administered shall have equipment for emergency resuscitation, suction, advanced airway equipment, positive pressure ventilation and supplemental oxygen in working order. Emergency medical equipment and supplies are immediately available and customized to the type of sedation being performed and the age and medical condition of the patient.

A person trained in airway management/anaesthesiologist shall be available in the organisation so that the person can rush to the area.

Standard

COP.13.

Anaesthesia services are provided in a consistent and safe manner.

Objective Elements

Commitment a. Anaesthesia services are provided in a consistent manner.*

Interpretation: Written guidance based on standard treatment guidelines/sound clinical practices govern anaesthesia services across the organisation. The organisation shall document the indications, the type of anaesthesia and procedure for the same. For the definition of the “anaesthesia” refer to the glossary.

The standard is not applicable to local anaesthesia.

CORE

- b. The pre-anaesthesia assessment results in the formulation of an anaesthesia plan which is documented.**

Interpretation: Patients for anaesthesia have a pre-anaesthesia assessment by a qualified anaesthesiologist. This shall be done before the patient is wheeled into the OT complex. It shall be applicable for both routine and emergency cases. The pre-anaesthesia assessment may even be carried out before admission in case of elective surgeries or shortly before the surgical procedure, as in emergency and obstetric patients.

The plan shall mention the pre-medications, type of anaesthesia, anaesthesia agent and dose, special requirements and anticipated post-anaesthesia care including post-operative pain management where appropriate. The anaesthesiologist shall review the medication the patient is currently taking.

The anaesthesia plan is documented in the patient's medical record.

Commitment

- c. A pre-induction assessment is performed and documented.**

Interpretation: A pre-induction assessment shall be done by an anaesthesiologist to re-evaluate patients immediately before the induction of anaesthesia. Any changes to the anaesthesia plan shall be documented. When anaesthesia needs to be provided on an urgent basis, the pre-anaesthesia assessment and pre-induction assessment may be performed immediately one-after-another, or simultaneously, but shall be documented separately in the patient's medical record.

Commitment

- d. The anaesthesiologist obtains informed consent for administration of anaesthesia.**

Interpretation: Patient and/or the family are educated on the risks, benefits, and alternatives of anaesthesia by the anaesthesiologist. The anaesthesia consent shall be separate from the surgery consent.

Refer to PRE.4.e.

CORE

- e. During anaesthesia, monitoring includes a regular recording of temperature, heart rate, cardiac rhythm, respiratory rate, blood pressure, oxygen saturation and end-tidal carbon dioxide.**

Interpretation: The parameters stated in the objective element shall be monitored and documented.

In the case of regional anaesthesia, instead of end-tidal carbon dioxide, the adequacy of ventilation shall be evaluated by continual observation of qualitative clinical signs.

Also, certain other parameters may be monitored on a case-to-case basis.

The cardiac rhythm is monitored on a monitor during the procedure, and the same need not be documented. However, in the case of rhythm abnormalities, the same shall be documented.

Anaesthesiologist shall be present throughout the procedure.

The results of monitoring are documented in the patient's medical record.

Commitment f. Patient's post-anaesthesia status is monitored and documented.

Interpretation: This shall be done in the recovery area/OT and at least include monitoring of vitals till the patient recovers completely from anaesthesia and shall be done by an anaesthesiologist. If the patient's condition is unstable, and he/she requires ICU care, the same shall be monitored there. When the patient is transferred directly from the OT to ICU, monitoring and documentation shall be the same as would be required in the recovery room.

Commitment g. The anaesthesiologist applies defined criteria to transfer the patient from the recovery area. *

Interpretation: The organisation documents these criteria which shall be based on physiologic parameters and in consonance with sound clinical practices.

Commitment h. The type of anaesthesia and anaesthetic medications used are documented in the patient record.

Interpretation: It shall have the name of the anaesthesiologist who performed the procedure and also the names of individuals (with their designation) who helped in the procedure. The type of anaesthesia and medications used during anaesthesia shall be documented in the anaesthesia chart. The documentation shall have the date, time and signature of the author for identification of the entry.

Refer to IMS.3.e. and IMS.3.f.

Commitment i. Procedures shall comply with infection prevention and control guidelines to prevent cross-infection between patients.

Interpretation: The guidelines shall be documented either separately or as a part of the infection prevention and control manual. This could include management of circuits, infection prevention and control measures during administration etc.

Refer to IPC.3.

Achievement j. Intra-operative adverse anaesthesia events are recorded and monitored.

Interpretation: Intra-operative adverse anaesthesia events are documented and monitored to take corrective and preventive action. At the outset, the organisation shall define the various intra-operative adverse anaesthesia events. These essentially are adverse events following the administration of anaesthesia. For example, cardiac arrest, failure of anaesthesia gas, a slip of the endotracheal tube, air embolism etc. The hospital shall have a mechanism to ensure that all adverse events are captured. It could do the same by incorporating in the anaesthesia record a heading for the same.

Standard

COP.14.

Surgical services are provided in a consistent and safe manner.

Objective Elements

Commitment a. Surgical services are provided in a consistent and safe manner. *

Interpretation: Written guidance based on standard treatment guidelines/sound clinical practices govern the provision of surgical services. This shall include the list of surgical procedures as well as the competency level for performing these procedures.

- Commitment b. Surgical patients have a pre-operative assessment, a documented pre-operative diagnosis, and pre-operative instructions are provided before surgery.**

Interpretation: Patients undergoing surgery are assessed pre-operatively, a pre-operative diagnosis is made, and pre-operative instructions documented. The relevant pre-operative instructions are provided to all concerned, including patient/family. This shall apply to all elective cases and whenever possible, to emergency cases. This shall be done by the operating surgeon or a doctor member of the operating team.

- Commitment c. Informed consent is obtained by a surgeon before the procedure.**

Interpretation: The consent shall be taken by the operating surgeon or a doctor member of the operating team. In case if there is a change in clinical status/expected outcomes after the consent, but before the surgery, the same is explained to the patient/family and is documented. In case, a new and/or an additional procedure that was not planned or for which an explicit consent had been taken before the surgery, a fresh consent needs to be taken for the same except in the case of life-saving procedures.

Refer to PRE.4.e.

- CORE d. Care is taken to prevent adverse events like the wrong site, wrong patient and wrong surgery. ***

Interpretation: Written guidance shall be available for preventing adverse events like the wrong site, wrong patient, wrong surgery by a suitable mechanism. The organisation shall be able to demonstrate methods to prevent these events, for example identification tags, badges, cross-checks, time-outs etc. Marking the surgical/invasive site involves the patient and is done with an instantly recognizable and unambiguous mark. The mark shall be consistent throughout the organisation. The site is marked in all cases involving laterality, multiple structures (fingers, toes, lesions), or multiple levels (spine). The surgical/invasive procedure site marking is done by the person or member of the team who will perform the procedure. This person will do the entire surgical/invasive procedure and remain with the patient throughout the entire procedure. The site marking may take place any time before the surgical/invasive procedure begins, as long as the patient is actively involved in the site marking whenever possible. The mark shall be visible in the procedural field when prepping and draping is complete.

The time-out shall be held immediately before the start of the procedure with all team members present. During the time-out, the team agrees on the correct patient identity, correct procedure to be done, correct surgical procedure site.

Commitment e. An operative note is documented before transfer out of patient from recovery.

Interpretation: This note provides information about the procedure performed, post-operative diagnosis and the status of the patient before shifting and shall be documented by the surgeon/doctor member of the operating team. If it is documented by a person other than the chief operating surgeon, the same shall be countersigned by the chief surgeon. At a minimum, it shall include the surgery performed, name of the surgeon(s), name of anaesthesiologist(s), post-operative diagnosis, peri-operative complications, amount of blood loss, salient steps of the procedure and the key intra-operative findings.

Commitment f. Post-operative care is guided by a documented plan.

Interpretation: The plan shall address, as required advice on IV fluids, medication, care of the wound, nursing care, observing for any complications, etc. This plan shall be documented by the operating surgeon or member of the operating team.

Commitment g. Patient, personnel and material flow conform to infection prevention and control practices.

Interpretation: The layout/practices of the theatre shall be such that the mix of sterile and unsterile materials is prevented. It is preferable to have separate closed trolleys for sterile and unsterile goods.

Commitment h. Appropriate facilities, equipment, instruments and supplies are available in the operating theatre.

Interpretation: The organisation shall ensure that the operation theatre complex has facilities for pre-op holding area, changing rooms, hand-washing, operating rooms, waiting area for relatives, storage area, collection area for waste and linen

and recovery room (where applicable). In addition to the equipment required for anaesthesia and surgery, there shall be equipment for resuscitation, radiation protection (where applicable) etc. Instruments shall be in a working condition. The organisation shall have a mechanism to verify the same. This could be obtained through feedback from surgeons/anaesthesiologists. The supplies to the OT are commensurate to the scope and complexities of the surgery.

Achievement i. The organisation shall implement a quality assurance programme. *

Interpretation: The written guidance for quality assurance could be developed individually, or it could be a part of the organisation's overall quality-improvement programme. The organisation shall monitor care-related outcomes. For example, intra-operative mishaps such as cautery burns, patient fall, position related nerve injuries; and peri-operative events including surgical site infections, deep venous thrombosis, etc. The organisation's quality assurance programmes shall also include aspects like pre-operative preparation, antimicrobial prophylaxis, adherence to set procedure(s) to prevent adverse events, etc.

Achievement j. The quality assurance programme includes surveillance of the operation theatre environment. *

Interpretation: Surveillance activities shall include the daily monitoring of humidity, pressure differential and temperature and at least six-monthly monitoring of the integrity of the filter. For air-conditioning of OT, refer to the guidelines issued by NABH.

Standard

COP.15. The organ transplant programme is carried out safely.

Objective Elements

CORE

- a. **The organ transplant programme shall be in consonance with the legal requirements and shall be conducted ethically.**

Interpretation: The organisation shall ensure that the required regulatory licences are in place and adhere to the requirements of 'The Transplantation of Human Organs Act'. The organisation also ensures that the relevant medical professionals have been permitted by the appropriate authorities to participate in the organ transplant programme. The organisation shall have a designated transplant coordinator. The organisation shall ensure that the requisite reports are submitted to the appropriate authorities as per the defined timeframe.

- Commitment** b. **Care of transplant patients is guided by clinical practice guidelines. ***

Interpretation: Written guidance based on standard treatment guidelines/sound clinical practices govern the care of transplant patient, which include an indication for transplant, recipient fitness, donor fitness, education and consent process, organ recovery and ensuring organ compatibility. The organ transplant program has documented organ-specific clinical practice guidelines for the pre-transplant, transplant, and discharge phase of transplantation. Individualized care plans are developed to guide the care of transplant patients. It is preferable that the organisation encourages departments to develop care pathways, which help guide these patients. A multidisciplinary team comprising of nurses, dietician, rehabilitation services, psychologist and/or medical social worker and infection prevention and control specialist is in place. It also ensures availability of other resources such as consumables, pharmaceutical supplies and other diagnostic requirements specifically required for transplant.

- Commitment** c. **The organisation ensures education and counselling of recipient and donor through trained / qualified counsellors before organ transplantation.**

Interpretation: The organisation ensures that qualified/trained counsellors are available to counsel the recipient and donor about the proposed organ donation. Doctors and counsellors educate and counsel the recipient and donor on the benefits and possible risks to make informed decisions. The recipients are also educated about immunosuppression (benefits and risks) and the required monitoring and follow up. Counselling can also be done during the interview by the Local Authorisation Committee of the organisation. Evidence of counselling is recorded and maintained in the requisite statutory formats apart from the organisation's individual needs.

CORE

- d. **The organisation shall take measures to create awareness regarding organ donation.**

Interpretation: This would be applicable for all organisations irrespective of whether organ transplantation is carried out or not. This could be in the form of standees, hand-outs, etc. The myths of the public regarding organ donation shall be addressed. Whenever a patient is declared brain dead, the family are also counselled on organ transplant where appropriate. They shall be educated in a very sensitive and courteous manner.

Standard

COP.16.

The organisation identifies and manages patients who are at high-risk of morbidity / mortality.

Objective Elements

CORE

- a. **The organisation identifies and manages vulnerable patients. ***

Interpretation: Written guidance for Identification and management of vulnerable patients is developed in consonance with statutory requirements, national and international guidelines.

It shall include (but not limited to) elderly, children, differently-abled and/or mentally challenged, patient with risk of suicide, self-harm, comatose, critically ill, patients under sedation and anaesthesia, pregnant women, patients on dialysis, patients receiving chemotherapy, immunosuppressed patients, etc. The guidance shall state who is responsible for identifying these patients, risk management in these patients and monitoring of these patients (at least twice a day).

The guidance shall include how informed consent is obtained from a vulnerable patient, and from the family or legal representative of a patient incapable of making an independent decision.

Refer to PRE.4.d.

The care is organised and delivered in accordance with written guidance.

Refer to the glossary for a definition of "vulnerable patient".

CORE**b. The organisation identifies and manages patients who are at a risk of fall.***

Interpretation: A validated tool shall be used for the assessment of the risk of fall. Patients found at a risk of fall shall be managed according to written guidance.

A good guide is 'Universal fall precautions'.

CORE**c. The organisation identifies and manages patients who are at risk of developing / worsening of pressure ulcers.***

Interpretation: A validated tool shall be used for assessment of the risk of pressure ulcers. For example, the Braden scale, the European and US National pressure ulcer advisory panels (EPUAP and NPUAP) staging systems to look for worsening of pressure ulcers. Patients found at risk of pressure ulcers shall be managed according to written guidance.

CORE**d. The organisation identifies and manages patients who are at risk of developing deep vein thrombosis.***

Interpretation: A validated tool shall be used for assessment of the risk of development of deep vein thrombosis. For example, Well's criteria. Patients found at risk of deep vein thrombosis shall be managed according to written guidance.

Commitment**e. The organisation identifies and manages patients who need restraints.***

Interpretation: Restraints include physical and chemical restraints. Written guidance is used to identify patients who need restraints and provide care to them. The organisation may minimise the use of restraints wherever possible keeping in view patient safety. The written guidance shall incorporate the suggested situations where restraints could be used. It shall also specify as to who can authorise the use of restraints, the frequency of monitoring these patients and the validity of restraint orders. The need for restraints is regularly reassessed, and the least invasive restraint is selected if required. The rationale for using restraints is explained to the family, and consent is obtained where appropriate (as directed by statutory requirements). These patients shall be monitored more frequently for the complications that the restraints may cause. For example, neurovascular deficit. When restraints are used, the following shall be documented in the medical record: the reason for using restraints and the time frame during which restraints were used.

Standard

COP.17.

Pain management for patients is done in a consistent manner.

Objective Elements

Commitment a. Patients in pain are effectively managed. *

Interpretation: Written guidance based on sound clinical practices govern the care of patients in pain.

The patients in pain receive care according to pain management guidelines and according to patient goals for pain management.

It shall include as to how patients are screened for pain, the mechanism to ensure that a detailed pain assessment is done (when necessary), pain mitigation techniques and monitoring.

Commitment b. Patients are screened for pain.

Interpretation: Patients entering the hospital shall be screened for pain. Pain shall be considered the fifth vital sign. Screening could be done by incorporating a yes/no question for pain in the initial assessment. A screening for pain may consist of one or more simple questions that can be asked during the assessment process and documented by the doctor or nurse in the medical record. An OPD patient who provided positive answers to the organization's pain screening questions could indicate the need for a more in-depth assessment of the patient's pain.

Commitment c. Patients with pain undergo detailed assessment and periodic re-assessment.

Interpretation: A detailed pain assessment is done when the pain is the predominant (or one of the main) symptom(s). For example, cancer pain, neuralgia and arthralgia. This does not include chest pain due to angina or where the aetiology of pain is physiological like labour pain. A validated tool shall be used for assessment of pain in various types of patients – neonates, paediatrics, adults, patients on ventilators, etc. It shall be done for all post-operative patients. The pain assessment shall include the intensity of the pain (can be done using an age-appropriate validated pain rating scale), pain character, frequency, location, duration and referral and/or radiation. The assessment shall be done in an objective manner so that it facilitates regular reassessment.

- Commitment d. Pain alleviation measures or medications are initiated and titrated according to the patient's need and response.**

Interpretation: Based on the assessment of pain and the underlying conditions, pain alleviation methods or medication are initiated for the patient. Pain management includes non-pharmacological, medical, surgical and anaesthetic techniques as per WHO pain ladder scale. Subsequently, the patient is monitored for response to pain alleviation methods. Based on the response, measures and medication are duly titrated. In case the hospital does not have facilities for further pain management, it could refer such patients to centres specialising in pain management.

Standard

COP.18.

Rehabilitation services are provided to the patients in a safe, collaborative and consistent manner.

Objective Elements

- Commitment a. Scope of the rehabilitation services at a minimum is commensurate to the services provided by the organisation.**

Interpretation: Rehabilitation services include physiotherapy, occupational therapy, speech therapy, clinical psychology etc. The organisation shall ensure the availability of rehabilitation services (at least basic) commensurate with its scope of the services. For example, an organisation providing neurological services shall have a neuro-rehabilitation service. In case the organisation does not have a facility for advanced rehabilitation, it could refer its patients to a centre with a relevant facility.

- Commitment b. Rehabilitation services are provided in a consistent manner.**

Interpretation: The organisation shall provide rehabilitation services as per written guidance in a consistent manner by adapting/adopting standard treatment guidelines/sound clinical practices.

Commitment c. Care providers collaboratively plan rehabilitation services.

Interpretation: Care providers include treating doctor, a rehabilitation therapist and other professional experts.

Commitment d. There are adequate space and equipment to provide rehabilitation.

Interpretation: Space and equipment shall be as per the scope of rehabilitation services provided. This could be based on guidance from professional bodies/documentated literature. Equipment for resuscitation shall be available in these areas.

Commitment e. Care is guided by functional assessment and periodic re-assessments which are done and documented.

Interpretation: The assessments can be done using relevant functional assessment scales. This shall be done by a qualified individual(s).

Commitment f. Care is provided adhering to infection prevention and control and safety practices.

Interpretation: Staff providing rehabilitation must adhere to infection prevention and control practices and safety guidelines defined by the organisation. For example, safe practices include ensuring that when using hot wax, there are no burns to the patient.

Refer to IPC.3.

Excellence g. Care pathways are developed, implemented, and reviewed periodically.

Interpretation: Care pathways shall be based on evidence and/or sound clinical practices. These provide the team, detailed guidance at various stages of rehabilitation. Development of care pathways is a continuous process.

Standard

COP.19.

Nutritional therapy is provided to patients consistently and collaboratively.

Objective Elements

Commitment a. Patients admitted to the organisation are screened for nutritional risk. *

Interpretation: Nutritional screening is done by the caregiver (doctor or nurse). Besides clinical assessment, nutritional screening may also be done using a validated tool such as Nutritional Risk Screening (NRS) malnutrition screening tool-2002), mini nutritional assessment- short-form, malnutrition universal screening tool (MUST) for adults, The Global leadership initiative on malnutrition (GLIM) criteria-2019, simple two-part tool, etc.

Commitment b. Nutritional assessment is done for patients found at risk during nutritional screening.

Interpretation: A dietician shall do a nutritional assessment for all patients found at risk during nutritional screening. It is preferable that nutritional assessment is done using a validated tool(s). The assessment shall include at a minimum, planning, preparation and distribution of diet, to the patient.

Commitment c. The therapeutic diet is planned and provided collaboratively.

Interpretation: The nutritional assessment shall result in the formulation of a therapeutic diet. The dietician shall ensure that this is planned in consultation with the treating doctor and the patient/patient's relative after taking into regard the patient's food allergies, food habits (veg/non-veg) and likes and dislikes. The process ensures that the patient receives food as per the diet order.

Commitment d. Patients receive food according to the written order for the diet.

Interpretation: Patients receive food according to their clinical needs. Written order could be either normal or therapeutic diet. Order shall be written in a uniform location in the medical record. Normal diet is provided to patients who are found to be at no risk during nutritional screening. In case of a therapeutic diet, dietician shall prepare this in the form of a diet sheet and patient shall receive food accordingly. Examples of therapeutic diets include diabetic diet, high protein diet, total parenteral nutrition, etc.

- Commitment e. When families provide food, they are educated about the patient's diet limitations.**

Interpretation: The dietician/nurse shall ensure education during planning. When family provides food, the dietician shall monitor compliance with the prescribed diet.

Standard

COP.20.

End-of-life-care is provided in a compassionate and considerate manner.

Objective Elements

- Commitment a. End-of-life care is provided in a consistent manner in the organisation. ***

Interpretation: The organisation develops written guidance for End-of-Life Care based on good practices (National and International) and in accordance with the law of the land.

End-of-life care provided by the organisation includes:

- providing appropriate pain, rehabilitative and palliative care according to the wishes of the family and patient;
- sensitively addressing such issues as autopsy and organ donation;
- respecting the patient's values, religion, and cultural preferences;
- involving the patient and family in all aspects of care; and
- responding to the psychological, emotional, spiritual, and cultural concerns of the patient and family (where possible).

The written guidance shall also incorporate requirements of objective element "c" and shall be prepared to keep in mind objective element "b".

Refer to the glossary for a definition of "end-of-life".

- Achievement b. A multi-professional approach is used to provide end-of-life care.**

Interpretation: The team could include a doctor, nurse, clinical psychologist, rehabilitation expert etc. It is preferable that someone trained in palliative care heads the team.

Commitment c. End-of-life care is in consonance with the legal requirements.

Interpretation: Decisions like “Do not resuscitate/Do not intubate/Allow natural death etc.” shall be only as per the statutory laws and within the guidelines framed by the legal system. For example: ICMR Do not Resuscitate Guideline, Supreme Court directives 2023 on Advanced Directives and Living Will.

Commitment d. End-of-life care also addresses the identification of the unique needs of such patient and family.

Interpretation: The psychological, emotional, cultural, religious and socio-cultural beliefs of patients/family shall be addressed and respected. Staff are educated about the unique needs of the patients and their families at the end of life.

Commitment e. Symptomatic treatment is provided and where appropriate measures are taken for the alleviation of pain.

Interpretation: The emphasis shall be on providing symptomatic treatment of such patients and to prevent complications to the possible extent. The patient and/or family shall be involved while taking all such decisions. To the extent possible (and as per the law), the patient and/or family's choices shall be respected.

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